

**STELARA (ustekinumab)
IMULDOSA (ustekinumab-srlf)
OTULFI (ustekinumab-aaaz)
PYZCHIVA (ustekinumab-ttwe)
SELARSDI (ustekinumab-aekn)
STEQEYMA (ustekinumab-stba)
WEZLANA (ustekinumab-auub)
YESINTEK (ustekinumab-kfce)
ustekinumab-aekn (unbranded Selarsdi)
ustekinumab-ttwe (unbranded Pyzchiva)**

Line(s) of Business:
HMO; PPO; QUEST

Original Effective Date:
10/01/2020
Current Effective Date:
04/08/2025

POLICY

A. INDICATIONS

The indications below including FDA-approved indications and compendial uses are considered a covered benefit provided that all the approval criteria are met and the member has no exclusions to the prescribed therapy.

FDA-Approved Indications

- Moderate to severe plaque psoriasis (PsO) in patients 6 years and older who are candidates for phototherapy or systemic therapy
- Active psoriatic arthritis (PsA) in patients 6 years and older
- Moderately to severely active Crohn's disease (CD) in adults
- Moderately to severely active ulcerative colitis (UC) in adults

Compendial Uses

- Immune checkpoint inhibitor-related toxicity

B. PRESCRIBER SPECIALTIES

The medication must be prescribed by or in consultation with one of the following:

1. Dermatologist: plaque psoriasis and psoriatic arthritis
2. Rheumatologist: psoriatic arthritis
3. Gastroenterologist: Crohn's disease, ulcerative colitis and immune checkpoint inhibitor-related toxicity
4. Hematologist or oncologist: immune checkpoint inhibitor-related toxicity

C. CRITERIA FOR INITIAL APPROVAL

1. Plaque psoriasis (PsO)

- a. Authorization of 12 months may be granted for members 6 years of age and older who have previously received a biologic or targeted synthetic drug (e.g., Sotyktu, Otezla) indicated for treatment of moderate to severe plaque psoriasis.
- b. Authorization of 12 months may be granted for members 6 years of age and older for treatment of moderate to severe plaque psoriasis when any of the following criteria is met:
 - i. Crucial body areas (e.g., hands, feet, face, neck, scalp, genitals/groin, intertriginous areas) are affected.
 - ii. At least 10% of body surface area (BSA) is affected.
 - iii. At least 3% of body surface area (BSA) is affected and the member meets either of the following criteria:
 - a. Member has had an inadequate response or intolerance to either phototherapy (e.g., UVB, PUVA) or pharmacologic treatment with methotrexate, cyclosporine, or acitretin.
 - b. Member has a clinical reason to avoid pharmacologic treatment with methotrexate, cyclosporine, and acitretin (see Appendix).

2. Psoriatic arthritis (PsA)

- a. Authorization of 12 months may be granted for members 6 years of age or older who have previously received a biologic or targeted synthetic drug (e.g., Rinvoq, Otezla) indicated for active psoriatic arthritis.
- b. Authorization of 12 months may be granted for members 6 years of age or older for treatment of active psoriatic arthritis when either of the following criteria is met:
 - i. Member has mild to moderate disease and meets one of the following criteria:
 - a. Member has had an inadequate response to methotrexate, leflunomide, or another conventional synthetic drug (e.g., sulfasalazine) administered at an adequate dose and duration.
 - b. Member has an intolerance or contraindication to methotrexate or leflunomide (see Appendix), or another conventional synthetic drug (e.g., sulfasalazine).
 - c. Member has enthesitis.
 - ii. Member has severe disease.

3. Crohn's disease (CD)

Authorization of 12 months may be granted for adult members for treatment of moderately to severely active Crohn's disease.

4. Ulcerative colitis (UC)

Authorization of 12 months may be granted for adult members for treatment of moderately to severely active ulcerative colitis.

5. Immune checkpoint inhibitor-related toxicity

Authorization of 6 months may be granted for the treatment of immune checkpoint inhibitor-related diarrhea or colitis when the member has experienced an inadequate response, intolerance, or contraindication to infliximab or vedolizumab.

D. CONTINUATION OF THERAPY

1. No previous authorization/precertification:

All members (including new members and members currently receiving treatment without prior authorization) must meet criteria for initial approval in section C.

2. Reauthorization:

A. Plaque psoriasis (PsO)

Authorization of 12 months may be granted to members 6 years of age and older who were previously authorized by HMSA/CVS for the requested drug and achieve or maintain a positive clinical response as evidenced by low disease activity or improvement in signs and symptoms of the condition when either of the following is met:

1. Reduction in body surface area (BSA) affected from baseline
2. Improvement in signs and symptoms from baseline (e.g., itching, redness, flaking, scaling, burning, cracking, pain)

B. Psoriatic arthritis (PsA)

Authorization of 12 months may be granted to members 6 years of age and older who were previously authorized by HMSA/CVS for the requested drug and who achieve or maintain a positive clinical response as evidenced by low disease activity or improvement in signs and symptoms of the condition when there is improvement in any of the following from baseline:

1. Number of swollen joints
2. Number of tender joints
3. Dactylitis
4. Enthesitis
5. Skin and/or nail involvement
6. Functional status
7. C-reactive protein (CRP)

C. Crohn's Disease (CD)

1. Authorization of 12 months may be granted for adult members who were previously authorized by HMSA/CVS and who achieve or maintain remission.
2. Authorization of 12 months may be granted to adult members who were previously authorized by HMSA/CVS for the requested drug and who achieve or maintain a positive clinical response as evidenced by low disease activity or improvement in signs and symptoms of the condition when there is improvement in any of the following from baseline:
 - i. Abdominal pain or tenderness
 - ii. Diarrhea
 - iii. Body weight
 - iv. Abdominal mass
 - v. Hematocrit
 - vi. Appearance of the mucosa on endoscopy, computed tomography enterography (CTE), magnetic resonance enterography (MRE), or intestinal ultrasound
 - vii. Improvement on a disease activity scoring tool (e.g., Crohn's Disease Activity Index [CDAI] score)

D. Ulcerative colitis

1. Authorization of 12 months may be granted for adult members who were previously authorized by HMSA/CVS and who achieve or maintain remission.
2. Authorization of 12 months may be granted for adult members who were previously authorized by HMSA/CVS for the requested drug and who achieve or maintain a positive clinical response as evidenced by low disease activity or improvement in signs and symptoms of the condition when there is improvement in any of the following from baseline:

- i. Stool frequency
- ii. Rectal bleeding
- iii. Urgency of defecation
- iv. C-reactive protein (CRP)
- v. Fecal calprotectin (FC)
- vi. Appearance of the mucosa on endoscopy, computed tomography enterography (CTE), magnetic resonance enterography (MRE), or intestinal ultrasound
- vii. Improvement on a disease activity scoring tool (e.g., Ulcerative Colitis Endoscopic Index of Severity [UCEIS], Mayo score)

E. Immune checkpoint inhibitor-related toxicity

All members requesting authorization for continuation of therapy must meet all initial authorization criteria.

F. OTHER

For all indications: Member has had a documented negative tuberculosis (TB) test (which can include a tuberculosis skin test [TST], an interferon-release assay [IGRA])* within 6 months of initiating therapy for persons who are naïve to biologic drugs or targeted synthetic drugs associated with an increased risk of TB.

* If the screening testing for TB is positive, there must be further testing to confirm there is no active disease (e.g., chest x-ray). Do not administer the requested medication to members with active TB infection. If there is latent disease, TB treatment must be started before initiation of the requested medication.

For all indications: Member cannot use the requested medication concomitantly with any other biologic drug or targeted synthetic drug for the same indication.

G. APPENDIX

Examples of Clinical Reasons to Avoid Pharmacologic Treatment with Methotrexate, Cyclosporine Acitretin, or Leflunomide.

1. Clinical diagnosis of alcohol use disorder, alcoholic liver disease or other chronic liver disease
2. Drug interaction
3. Risk of treatment-related toxicity
4. Pregnancy or currently planning pregnancy
5. Breastfeeding
6. Significant comorbidity prohibits use of systemic agents (e.g., liver or kidney disease, blood dyscrasias, uncontrolled hypertension)
7. Hypersensitivity
8. History of intolerance or adverse event

H. DOSAGE AND ADMINISTRATION

Approvals may be subject to dosing limits in accordance with FDA-approved labeling, accepted compendia, and/or evidence-based practice guidelines.

I. ADMINISTRATIVE GUIDELINES

Precertification is required. Please refer to the [HMSA Medical Specialty Drug policy website](#) for the fax form.

J. IMPORTANT REMINDER

The purpose of this Medical Policy is to provide a guide to coverage. This Medical Policy is not intended to

dictate to providers how to practice medicine. Nothing in this Medical Policy is intended to discourage or prohibit providing other medical advice or treatment deemed appropriate by the treating physician.

Benefit determinations are subject to applicable member contract language. To the extent there are any conflicts between these guidelines and the contract language, the contract language will control.

This Medical Policy has been developed through consideration of the medical necessity criteria under Hawaii's Patients' Bill of Rights and Responsibilities Act (Hawaii Revised Statutes §432E-1.4), generally accepted standards of medical practice and review of medical literature and government approval status. HMSA has determined that services not covered under this Medical Policy will not be medically necessary under Hawaii law in most cases. If a treating physician disagrees with HMSA/CVS's determination as to medical necessity in a given case, the physician may request that HMSA reconsider the application of the medical necessity criteria to the case at issue in light of any supporting documentation.

K. REFERENCES

1. Stelara [package insert]. Horsham, PA: Janssen Biotech, Inc.; August 2022.
2. Imuldosa [package insert]. Raleigh, NC: Accord BioPharma Inc.; October 2024.
3. Otulfi [package insert]. Lake Zurich, IL: Fresenius Kabi USA, LLC; September 2024.
4. Pyzchiva [package insert]. Princeton, NJ: Sandoz Inc.; December 2024.
5. Selarsdi [package insert]. Leesburg, VA: Alvotech USA Inc.; October 2024.
6. Steqeyma [package insert]. Jersey City, NJ: Celltrion USA, Inc.; December 2024.
7. ustekinumab-aekn [package insert]. Leesburg, VA: Alvotech USA Inc.; October 2024.
8. ustekinumab-ttwe [package insert]. Grand Cayman, Cayman Islands: Quallent Pharmaceuticals Health LLC; March 2025.
9. Wezlana [package insert]. Thousand Oaks, CA: Amgen Inc.; October 2023.
10. Yesintek [package insert]. Cambridge, MA: Biocon Biologics Inc.; November 2024.
11. Menter A, Korman NJ, Elmets CA, et al. Guidelines of care for the management of psoriasis and psoriatic arthritis. Section 6: Guidelines of care for the treatment of psoriasis and psoriatic arthritis: case-based presentations and evidence-based conclusions. *J Am Acad Dermatol*. 2011;65(1):137-174.
12. Gossec L, Baraliakos X, Kerschbaumer A, et al. European League Against Rheumatism (EULAR) recommendations for the management of psoriatic arthritis with pharmacological therapies: 2019 update. *Ann Rheum Dis*. 2020;79(6):700-712.
13. Gladman DD, Antoni C, Mease P, et al. Psoriatic arthritis: epidemiology, clinical features, course, and outcome. *Ann Rheum Dis* 2005;64(Suppl II):ii14–ii17.
14. Talley NJ, Abreu MT, Achkar J, et al. An evidence-based systematic review on medical therapies for inflammatory bowel disease. *Am J Gastroenterol*. 2011;106(Suppl 1):S2-S25.
15. Lichtenstein GR, Loftus Jr EV, Isaacs KI, et al. ACG Clinical Guideline: Management of Crohn's Disease in Adults. *Am J Gastroenterol*. 2018;113:481-517.
16. Menter A, Strober BE, Kaplan DH, et al. Joint AAD-NPF guidelines of care for the management and treatment of psoriasis with biologics. *J Am Acad Dermatol*. 2019;80(4):1029-1072.
17. Testing for TB Infection. Centers for Disease Control and Prevention. Retrieved on January 5, 2024 from: <https://www.cdc.gov/tb/topic/basics/risk.htm>.
18. Talley NJ, Abreu MT, Achkar J, et al. An evidence-based systematic review on medical therapies for inflammatory bowel disease. *Am J Gastroenterol*. 2011;106(Suppl 1):S2-S25.
19. Rubin DT, Ananthakrishnan AN, et al. 2019 ACG Clinical Guideline: Ulcerative Colitis in Adults. *Am J Gastroenterol*. 2019;114:384-413.
20. Feuerstein JD, Isaacs KL, Schneider Y, et al. AGA Clinical Practice Guidelines on the Management of Moderate to Severe Ulcerative Colitis. *Gastroenterology* 2020; 158:1450.

21. Singh JA, Guyatt G, Ogdie A, et al. 2018 American College of Rheumatology/National Psoriasis Foundation Guideline for the Treatment of Psoriatic Arthritis. *Arthritis Rheumatol*. 2019;71(1):5-32. doi:10.1002/art.40726.
22. Menter, A, Cordero, KM, Davis, DM, et al. Joint AAD-NPF guidelines of care for the management and treatment of psoriasis in pediatric patients. *J Am Acad Dermatol*. 2020;82(1):161-201.
23. Menter, A, Gelfand, JM, Connor, C, et al. Joint AAD-NPF guidelines of care for the management of psoriasis with systemic nonbiologic therapies. *J Am Acad Dermatol*. 2020;82(6): 1445-86.
24. Feuerstein JD, Ho EY, Shmidt E, et al. AGA Clinical Practice Guidelines on the Medical Management of Moderate to Severe Luminal and Perianal Fistulizing Crohn's Disease. *Gastroenterology* 2021; 160: 2496-2508.
25. The NCCN Drugs & Biologics Compendium® © 2024 National Comprehensive Cancer Network, Inc. Available at: <http://www.nccn.org>. Accessed January 15, 2024.
26. Coates LC, Soriano ER, Corp N, et al. Group for Research and Assessment of Psoriasis and Psoriatic Arthritis (GRAPPA): updated treatment recommendations for psoriatic arthritis 2021. *Nat Rev Rheumatol*. 2022;18(8):465-479.
27. NCCN Clinical Practice Guidelines in Oncology® (NCCN Guidelines®). Management of Immunotherapy-Related Toxicities. Version 1.2024. Available at: www.nccn.org. Accessed January 15, 2024.

Document History

05/2020	Adopt HMSA's Pharmacy Benefit Manager Standard Criteria
10/1/2020	Original Effective Date
02/2021	Annual Review: Updated continuation of therapy to align with HMSA's Pharmacy Benefit Manager standard
01/01/2022	Revision effective date
04/2022	Updated TB test requirements
09/01/2022	Revision effective date
09/2022	Annual review
01/2023	Updated compendial uses, added prescriber restrictions, and updated PsA criteria
04/01/2023	Revision effective date – annual review
09/2023	Annual review
10/01/2023	Off-cycle revision effective date
01/01/2024	Removed Medicare LOB for Final Rule
02/01/2024	Revision effective date – annual review
09/2024	Annual review
02/22/2025	Revision effective date
04/2025	Added biosimilars